Key Concepts from Potter & Perry and Lewis (Unit 4.2 Content)

Potter & Perry Chapter 26 – The Experience of Loss, Death, and Grief

- **Loss** is any actual or perceived removal of a person, object, role, function, idea, or environment that has meaning for an individual, leading to a grief response that is shaped by the person's values, relationships, and social–cultural context.[^1]
- Types of loss include **actual vs perceived** loss, **maturational** loss linked to normal life transitions, **situational** loss related to sudden external events, and multiple losses often seen with chronic illness, disability, or hospitalization.[^1]
- Losses may involve external objects, familiar environments, significant others, aspects of self (body part, function, or role), or life itself; the degree of grief depends on the value and attachment placed on what was lost.[^1]
- **Grief** is the total response (physical, emotional, cognitive, social, spiritual) to loss; **bereavement** is the state of having lost a significant other and living through grief and mourning.[^1]
- Grief is **individual and non-linear**; people oscillate between more intense and less intense periods rather than passing neatly through fixed stages, and many eventually adapt with new meanings and coping strategies.[^1]

Theories and Types of Grief

- Classic grief theories include Kübler-Ross's stages (denial, anger, bargaining, depression, acceptance), Bowlby and Parkes' attachment-based phases (numbness, yearning/protest, disorganization/despair, reorganization), the **dual-process model** (oscillation between loss-oriented and restoration-oriented coping), meaning-making models, and continuing bonds.[^1]
- **Normal (uncomplicated) grief** involves a range of feelings (sadness, anger, loneliness, guilt, relief), thoughts (preoccupation, disbelief), behaviours (crying, withdrawal), and physical sensations (tight chest, fatigue) that gradually become more manageable over time.[^1]
- **Anticipatory grief** occurs when loss is expected (for example with terminal illness or dementia) and caregivers begin grieving before death; it may help some adjust but can also increase anxiety, distress, and risk of complicated grief, so coping and supports must be assessed.[^1]
- **Complicated or prolonged grief** (including prolonged grief disorder) involves chronic, intense, and disabling grief reactions, persistent separation distress, and functional impairment that continue long after the loss, and typically require specialized assessment and intervention.[^1]

- **Disenfranchised grief** arises when a loss is not socially recognized or supported (for example, death by overdose, loss after divorce, health-care providers' grief over patients, or pet loss), limiting expression and support and increasing risk of complicated grief.[^1]

Death, "Good Death," and Types of Death

- Biological death is defined by lack of oxygen and may be categorized as **neurological death** (irreversible cessation of all brain function including brainstem) or **circulatory death** after cardiorespiratory arrest; social, psychological, and theological notions of death capture loss of social roles, internal acceptance of dying, and separation of body and soul.[^1]
- A "**good death**" is highly individualized but generally involves symptom control, a sense of closure and preparedness, preserved dignity, and feeling seen as a person who can still give to others; culture, spirituality, disease trajectory, and age shape what this looks like.[^1]
- A **Dying Person's Bill of Rights** emphasizes control, being treated as a living human being, having purpose, being able to express emotions and spirituality, having questions answered honestly, not dying alone, and being cared for by knowledgeable, caring people.[^1]

Factors Influencing Loss and Grief

- Responses to loss are influenced by development (e.g., children's limited understanding vs. older adults' multiple losses), prior experiences, coping skills, socioeconomic status, nature and visibility of the loss, quality of relationships, culture and ethnicity, and spiritual beliefs.[^1]
- Children need honest communication, safe spaces to express feelings, and opportunities to maintain connection and create memories; older adults may face multiple concurrent losses and increased risk of prolonged grief, especially in contexts like the COVID-19 pandemic.[^1]
- Cultural and spiritual contexts shape beliefs about death, acceptable expressions of grief, rituals, and decisions about life-sustaining treatments; culturally safe care is particularly emphasized for Indigenous peoples given colonial histories and systemic inequities.[^1]

Nursing Process for Loss, Death, and Grief

- **Assessment** focuses on the meaning of the loss, type and stage of grief, physical and psychological symptoms, coping strategies, supports, cultural and spiritual context, and the responses of both patient and family.[^1]
- Nurses should avoid assumptions about "how people should grieve," encourage narratives ("Tell me what this has been like for you"), watch for non-verbal cues, and differentiate normal from complicated grief.[^1]
- **Interventions** include therapeutic communication (presence, open-ended questions, validation), promoting hope, supporting existing coping, introducing new coping

approaches (e.g., journaling), coordinating interprofessional support, and facilitating a palliative approach and hospice palliative care when indicated.[^1]

- At end of life, nurses support symptom management, explain signs of impending death, assist with decision-making and advance care planning, support family caregivers (including respite, teaching, and preparation for death), and respect rituals and cultural practices.[^1]
- **Post-death care** includes respectful care of the body, support for family viewing and goodbye, attention to legal/documentation requirements, and appropriate communication and referral for bereavement support.[^1]
- Nurses themselves are vulnerable to grief and **bereavement overload**; debriefing, team support, stress-management, and self-care are essential for sustaining practice.[^1]

Potter & Perry Chapter 29 – Spirituality in Health and Healthcare

- **Spirituality** is a way of being that involves connectedness to self, others, nature, or a higher power, a sense of meaning or purpose, and transcendence beyond everyday concerns and suffering; it may or may not be linked to organized religion.[^1]
- **Religion** is an organized system of beliefs and practices centred on a deity or ultimate reality, typically expressed through community worship, rituals, and moral codes; people may be spiritual but not religious, religious and spiritual, or neither.[^1]
- **Faith** can refer to affiliation with a specific religion and/or a personal relationship with a higher power; it often provides a framework for meaning-making and coping.[^1]

Spirituality, Health, and Diversity in Canada

- Nursing has historical roots in religious and spiritual traditions (e.g., Catholic orders, Christian missionary nurses), and contemporary practice re-emphasizes spirituality as a dimension of holistic care.[^1]
- Canada is religiously and spiritually diverse, with declining affiliation with majority religions, growth of "nones," and increasing diasporic religions (e.g., Islam, Hinduism, Sikhism, Buddhism) due to immigration; Indigenous spiritualities are being reclaimed in the context of reconciliation.[^1]
- Spirituality and religion are recognized as **social determinants of health** linked to coping, resilience, belonging, and health behaviours; at the same time, people can experience discrimination based on religion or spirituality (for example, Islamophobia, Indigenous-specific racism).[^1]

Spirituality, Suffering, and Life Journey

- Illness and loss often raise existential questions and can create **spiritual suffering** (e.g., feeling punished by a higher power, abandoned, or struggling to find meaning) or spiritual transformation (greater connection, growth, and re-prioritization of values).[^1]
- Spiritual experiences and needs vary across contexts such as pediatrics, maternity, mental health, community care, long-term care, and palliative care; creative, developmentally

appropriate, and culturally safe approaches are needed in each.[^1]

- Spiritual practices (e.g., prayer, meditation, rituals, connection with land and community, sacred objects) can reduce stress and support coping; faith communities often provide social support and health-promoting activities.[^1]

Nursing and Spiritual Care

- Holistic nursing recognizes the unity of mind, body, spirit, and social context; spiritual care is viewed as an integral part of compassionate care, not an "add-on."[^1]
- Key **spiritual care competencies** include self-awareness and attitude, communication about spirituality, understanding spiritual dimensions of the nursing process, planning and implementing spiritual support, contributing to quality improvement, and knowing when to refer to spiritual care providers.[^1]
- Professional ethics require nurses to treat all patients with dignity and respect, avoid discrimination based on spiritual or religious beliefs, recognize power differentials, and refrain from misusing their position (e.g., proselytizing or imposing beliefs).[^1]

Understanding Patients' Spirituality

- Spiritual "assessment" is reframed as **understanding** patients' spirituality through conversation and relationship, focusing on what beliefs and practices matter for care, decision-making, and coping.[^1]
- Simple tools such as the **2Q-SAM** ("What's most important to you right now?" and "How can we help?") and structured models like **T.R.U.S.T.** and **FICA** (Faith, Importance, Community, Address in care) provide frameworks for exploring spirituality in a patient-centred way.[^1]
- Nurses should clarify religious or spiritual practices that have implications for care (e.g., dietary restrictions, sacred objects, clothing, prayer times, end-of-life rituals) while recognizing individual variation within traditions.[^1]

Offering Spiritual Support and Interprofessional Collaboration

- Spiritual practices that nurses may need to support include prayer, meditation, sacred readings, rituals, use of religious symbols, dietary observances, and connection with nature or community; respecting and safeguarding sacred items is essential.[^1]
- A **layered model of spiritual care** distinguishes primary spiritual care (all clinicians' responsibility for basic spiritual support and presence), supportive spiritual care (staff with additional training), and specialized spiritual care (chaplains, Elders, or faith leaders addressing deep spiritual distress).[^1]
- Nurses must know when and how to **refer** patients to spiritual care providers (e.g., in crisis, major loss, complex ethical decisions, or when spiritual struggles exceed the nurse's competence) and must seek consent before referral.[^1]

Nurse's Own Spirituality and Self-Care

- Reflective practice about one's own spirituality, biases, and beliefs (e.g., via guided questions in Box 29.3) helps prevent judgment and fosters authentic, ethical care.[^1]
- Practices such as mindfulness, reflective journalling, gratitude, and cultivating presence help nurses nurture their own spirituality, manage stress, and sustain compassion.[^1]
- Organizational supports (e.g., sacred spaces, staff debriefing, initiatives like "Compassion Carts" or "Lavender Rooms") promote staff spiritual well-being and resilience.[^1]

Lewis Chapter 2 – Cultural Competence and Health Equity in Nursing Care

- Culture shapes beliefs about health, illness, acceptable treatments, decision-makers, and expressions of distress; Canadian health care systems are rooted in a dominant white, Eurocentric biomedical model that may conflict with other healing systems.[^1]
- **Cultural competence and safety** involve recognizing power imbalances, racism, and colonial histories (especially for Indigenous peoples), engaging in self-reflection and cultural humility, and partnering with patients and communities in defining safe care.[^1]
- Nurses must consider social determinants of health (e.g., income, education, migration experiences, racism) and how these intersect with culture, spirituality, and religion to produce inequities in access, experiences, and outcomes.[^1]
- Many individuals draw on multiple **healing systems**, including Western medicine, Indigenous medicine, traditional Chinese medicine, Ayurveda, homeopathy, and complementary and alternative therapies; nurses should assess for all therapies used and support safe integration.[^1]
- Cultural norms influence **time orientation**, communication styles, personal space, family roles, attitudes to pain, decision-making, and help-seeking (e.g., relying on family or spiritual leaders rather than professionals); nurses need to explore patients' own beliefs rather than stereotyping.[^1]
- For Indigenous peoples, loss of land, language, and culture, residential schools, and ongoing racism have produced profound health inequities; cultural safety requires building trust, respecting Indigenous knowledge and spiritual practices, and addressing structural barriers.[^1]

Lewis Chapter 11 – Substance Use (Key Med-Surg Concepts)

> Note: The attached Unit-4.2 document does not detail Lewis Chapter 11; this section is based on Canadian med-surg chapter summaries for Lewis and related resources.[2][3]

- Substance use exists on a **continuum** from non-use to low-risk use to substance use disorder (SUD); not all use is problematic, but nurses must routinely assess use in all settings and identify harms.[^2]
- A **substance use disorder** is defined as a cluster of cognitive, behavioural, and psychological symptoms indicating impaired control, social impairment, risky use, and

pharmacological criteria (tolerance, withdrawal); severity guides individualized treatment planning.[^2]

- Commonly used substances include **alcohol, tobacco/nicotine, opioids, cannabis, stimulants, and sedatives**, each with characteristic acute effects, withdrawal syndromes, and long-term health consequences (e.g., liver disease, cardiovascular disease, overdose).[4][2]
- Risk and protective factors span genetics, trauma, family and social environment, concurrent mental illness, chronic pain, social determinants of health, stigma, and access to services; problematic use affects all populations but harms are concentrated where inequities exist.[^2]
- The current **opioid overdose crisis** in Canada has intensified since the COVID-19 pandemic, with high rates of morbidity and mortality; hospital settings are key points of contact for people with SUD.[^2]
- Nursing priorities include **non-stigmatizing, trauma- and harm-reduction-oriented care**, systematic screening and brief interventions, symptom management for intoxication/withdrawal, health teaching, motivational interviewing, and coordination with interprofessional and community resources.[3][2]

Lewis Chapter 13 – Palliative and End-of-Life Care (Key Med-Surg Concepts)

> Note: Specific Lewis Chapter 13 content is not fully reproduced in Unit-4.2; these key points synthesize standard Canadian palliative-care references and Lewis-based chapter summaries.[5][6] [7][1]

- **Palliative care** is an approach that improves quality of life for patients and families facing life-threatening illness through prevention and relief of suffering by early identification, assessment, and treatment of pain and other physical, psychosocial, and spiritual problems.[6][7]
- A **palliative approach** can be integrated early in the disease trajectory, alongside disease-modifying treatments, while **end-of-life care** focuses on comfort, preparation, and support when death is expected in the near future.[7][1]
- Goals of care include symptom control (e.g., pain, dyspnea, nausea, delirium), maintaining function and autonomy where possible, supporting decision-making and advance care planning, and addressing emotional, social, cultural, and spiritual needs.[5][1]
- Common physical manifestations of impending death include changes in senses, decreased intake, weakness, changes in breathing patterns (e.g., Cheyne-Stokes respirations, “death rattle”), altered circulation and skin, and changes in level of consciousness.[5][1]
- Nurses play a central role in **communication** (honest information, aligning care with patient values), **care coordination** across settings, support for family caregivers (education, respite, bereavement support), and advocacy for equitable access to palliative services.[6][7] [^1]

- Post-death, nursing care extends to respectful care of the body, facilitating cultural and spiritual rituals, supporting the family's grief, and ensuring appropriate documentation and follow-up/referrals.[7][1]

Lewis Chapter 24 – Nursing Management: Visual and Auditory Conditions (Selected Topics)

> Note: Detailed disease-specific content for Chapter 24 is not present in Unit-4.2; the following key points integrate standard Lewis med-surg outlines and question banks.[8][9] [10][11]

Cataracts

- A **cataract** is an opacity of the lens causing decreased vision, glare, and colour changes; it is commonly age-related but can be congenital, traumatic, or secondary to drugs or disease.[11][8]
- Management focuses on visual aids and, when impairment is significant, **surgical lens extraction with intraocular lens (IOL) implantation**; post-operative nursing care includes eye protection, avoiding activities that increase intraocular pressure, and recognizing signs of complications (e.g., severe pain, decreased vision).[10][11]

Age-Related Macular Degeneration (AMD)

- **AMD** is a degenerative condition of the macula leading to central vision loss; the dry form involves gradual atrophy, while wet AMD involves neovascularization and leakage.[^8]
- Risk factors include age, smoking, family history, and possible cardiovascular factors; management includes **AREDS-type vitamins** for some dry AMD and anti-VEGF injections and lifestyle modification for wet AMD; nurses emphasize smoking cessation and low-vision supports.[^8]

Glaucoma

- **Glaucoma** is a group of disorders characterized by optic nerve damage, often associated with increased intraocular pressure; open-angle glaucoma is chronic and progressive, while angle-closure glaucoma is acute and sight-threatening.[11][8]
- Classic teaching highlights gradual peripheral vision loss in open-angle disease and sudden eye pain, halos, and blurred vision in angle-closure; treatment includes **topical medications** (e.g., beta-blockers, prostaglandin analogues) and sometimes laser or surgical procedures.[10][11]
- Nursing care stresses adherence to life-long therapy, recognizing acute symptoms that require emergency care, and ensuring safe self-administration of drops.[^10]

Acute and Chronic Otitis Media

- **Otitis media** is inflammation/infection of the middle ear, often following upper respiratory infections; it can be acute (pain, fever, hearing changes) or chronic (recurrent infection, effusion, possible perforation and hearing loss).[^10]
- Management includes appropriate analgesia, antibiotics when indicated, and patient education about completing treatment, preventing recurrence, and monitoring for complications; in chronic cases, surgical interventions (e.g., tympanostomy tubes) may be considered.[^10]

Ménière's Disease and Benign Paroxysmal Positional Vertigo (BPPV)

- **Ménière's disease** involves episodic vertigo, fluctuating sensorineural hearing loss, tinnitus, and aural fullness related to endolymphatic hydrops; attacks can be disabling.[^10]
- Nursing care centers on safety during acute vertigo (fall prevention, bed rest), dietary management (e.g., sodium restriction as ordered), medications (e.g., vestibular suppressants), and coping with chronic symptoms.[^10]
- **BPPV** is characterized by brief episodes of vertigo triggered by head position changes due to displaced otoliths; it is typically diagnosed with positional tests and treated with canalith repositioning maneuvers (e.g., Epley).[^10]

Hearing Loss and Deafness

- Hearing loss ranges from mild to profound and may be **conductive, sensorineural, or mixed**; common causes include noise exposure, aging (presbycusis), ototoxic medications, infections, and congenital factors.[9][10]
- Nursing priorities include screening, referral for audiology and amplification (e.g., hearing aids, cochlear implants), communication strategies (face patient, speak clearly, use written aids, sighted-guide techniques for those with combined vision/hearing issues), and environmental modifications.[^10]
- Respect for Deaf culture and individualized communication preferences (e.g., sign language, interpreters, assistive devices) is essential for equitable, person-centred care.[^10]

References

1. Unit-4.2.pdf - c. Prepare to administer Sylvies prn medication because she appears to be in distress. d. Sit Sylvie...
2. Understanding Substance Use and Its Impact in Canada - Compressed Chapter notes from Lewis Med/Surg. Original title: Chapter 11 Substance Use. Academic yea...
3. Lewis's Medical-Surgical Nursing in Canada, 5th Edition - Updated Substance Use chapter covers stigmatizing behaviours by health care ... 11 Substance Use 12 ...

4. Medical-Surgical Nursing in Canada - Ch 11: Substance ... - The nurse is assessing a client who has a history of alcohol use. Which of the following. assessment...
5. Medsurg CH 13: Palliative Care Insights & Key NCLEX ... - Chapter 13: Palliative Care at the End of Life. Lewis: Medical-Surgical Nursing in Canada, 4th Canad...
6. A Palliative Approach to Care in the Last 12 Months of Life - The Registered Nurses' Association of Ontario (RNAO) is delighted to present the clinical best pract...
7. Palliative and End-of-Life Care - Palliative and end-of-life care includes both caregiver and bereavement support, and involves the "c...
8. Intermediate Med-Surg: Chapter 22 – Visual Problems and Eye ... - From normal anatomy to conditions like cataracts, glaucoma, macular degeneration, and retinal detach...
9. Lewis's medical–surgical nursing: Assessment and Management of ... - Outline the structures and functions of the visual and auditory systems. 2. Outline the physiologica...
10. NURS 24: Nursing Management of Visual and Auditory Problems - Chapter 24: Nursing Management: Visual and Auditory Problems. Lewis: Medical-Surgical Nursing in Can...
11. Cataracts, Glaucoma - Medical-Surgical - Nervous System - YouTube - Cataracts and glaucoma. The pathophysiology, risk factors, signs/symptoms, diagnosis, treatment for ...